

Family Medicine Assistant

Complete capability inventory · generated 2026-08-25 from the live application

What this document is for. Every feature the site currently ships, so you can decide what to keep and what to remove. Nothing here is aspirational — the list is generated directly from the application data, so it cannot drift from the code. Counts in the tables are structural (how many inputs, how many content blocks), not estimates.

25 Clinical calculators	33 Clinical guides	21 Prescription templates	10 Reference tools	10 Patient handouts
17 Note visit types				

1. Progress Note Assistant

A two-stage documentation tool. Stage one produces a form that adapts to the presenting complaint; stage two assembles the note in house format and returns teaching feedback with a rubric score. It runs entirely on the device — no API key, no network call, no patient data leaving the phone.

WHAT IT ENFORCES

- **Never invents clinical content.** Any required field left blank becomes a visible [TO COMPLETE: ...] placeholder, rendered in red, and the copy button stays locked until none remain.
- **Red flags are surfaced, never smoothed over.** Flags you entered raise an alert that needs an explicit acknowledge or dismiss, and that choice is logged. If your assessment already carries the flag, the tool says so instead of nagging.
- **Judgemental language is rewritten** before it reaches the record — "non-compliant" becomes "admits missing doses" — and you are told which word and why.
- **Patient identifiers are stripped.** Phone numbers, national IDs, MRNs and full dates of birth are removed. Age and sex only.
- **Every note ends with safety netting.** ER instructions and a follow-up interval, auto-suggested from the complaint if you leave them blank and flagged as auto-suggested so you review them.
- **Mean BP is computed** from your SBP and DBP rather than asked for.

FOUR MODES

Write a note	Pick a visit type, fill the adaptive form, get the note plus rubric and teaching	main path
Critique my note	Paste a note you wrote; returns the rubric, teaching, and a corrected Assessment section only	rewriting the whole note teaches nothing
Quick expand	Shorthand in, full note out, every unsupplied element left as a visible placeholder	fast path for busy clinics
Practice	Four worked scenarios with model answers and the rules behind each	difficulty adapts to your rubric history

RUBRIC (SCORED 0-2 EACH, 10 TOTAL, STORED ACROSS VISITS)

Problem list detail	Drug, dose, frequency and control status on every line
Problem-specific negatives	Red-flag screen completed and the specific negatives documented
Assessment actionable	Every line ends in an action and carries a number
Defensive documentation	Deliberate non-action recorded with its reason; disagreements in the four-part sequence
Safety netting	Specific ER instructions and a stated follow-up interval

VISIT TYPES (17)

VISIT TYPE	FIELDS / REQUIRED	RED-FLAG OPTIONS
Chronic disease follow-up	36 fields, 22 required	5
Routine visit / labs / refill	34 fields, 20 required	5
Palpitation	37 fields, 24 required	8
Chest pain	37 fields, 24 required	10
Headache	35 fields, 22 required	12
Low back pain	34 fields, 21 required	11
Dizziness / vertigo	36 fields, 23 required	7
Fatigue / tiredness	34 fields, 21 required	6
Sore throat / URTI	34 fields, 21 required	8
Urinary symptoms	35 fields, 22 required	7
Abdominal pain	34 fields, 21 required	8
Joint pain	34 fields, 21 required	7
Shortness of breath / cough	34 fields, 21 required	12
Comprehensive elderly visit · older adults	37 fields, 24 required	12
Falls assessment · older adults	37 fields, 24 required	16
Memory concern · older adults	36 fields, 23 required	12
Other complaint (generic)	34 fields, 21 required	6

Deciding on this feature. It is the only part of the site that produces an artefact for the medical record rather than a reference lookup, so it carries the most risk and the most benefit. It is also the largest single piece of code. If you drop it, everything else still works unchanged — it is fully self-contained in three files.

2. Clinical Calculators (25)

All compute live as you type, with no calculate button. Every result carries its interpretation band and the practical next step, not just a number.

CORE (3)

Body Mass Index	BMI, body surface area, ideal weight range	2 inputs
Estimated GFR	CKD-EPI 2021, race-free; stages and drug implications	4 inputs
Creatinine Clearance	Cockcroft-Gault, for drug dose adjustment	4 inputs

CARDIOVASCULAR (3)

ASCVD 10-Year Risk	Pooled Cohort Equations; statin intensity guidance	9 inputs
CHA₂DS₂-VASc	Stroke risk in atrial fibrillation	7 items
HAS-BLED	Bleeding risk on anticoagulation	9 items

INFECTION (2)

Centor / McIsaac	Probability of streptococcal pharyngitis	5 items
Wells Score (DVT)	Deep vein thrombosis probability	10 items

MENTAL HEALTH (2)

PHQ-9	Depression severity	9 items
GAD-7	Anxiety severity	7 items

LABORATORY (4)

HbA1c Converter	HbA1c to estimated average glucose	1 inputs
Corrected Calcium	Albumin-corrected calcium	2 inputs
FIB-4	Advanced fibrosis risk in fatty liver disease	4 inputs
Anion Gap	With albumin and glucose corrections	5 inputs

WOMEN'S HEALTH (1)

Gestational Age & EDD	From LMP, with antenatal milestones	2 inputs
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PREVENTION (1)

Pack-Years	Smoking burden and screening eligibility	2 inputs
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OLDER ADULTS (9)

Clinical Frailty Scale	CFS 1-9 — sets targets for every other decision	1 inputs
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Katz ADL	Basic functional independence	6 items
Mini-Cog	Three-minute cognitive screen	2 items
4AT	Delirium detection in two minutes	4 items
GDS-15	Geriatric Depression Scale	15 items
Anticholinergic Burden	Cumulative ACB score across the medication list	16 items
Falls Risk	STEADI screen and modifiable factors	12 items
Orthostatic Hypotension	Lying vs standing blood pressure	4 inputs
MNA-SF	Malnutrition screening	6 items

Validation note. The ASCVD calculator was checked against the published reference cases for all four sex and ethnicity combinations and matches. eGFR uses the 2021 race-free CKD-EPI equation.

3. Clinical Guides (33)

Short reference summaries: diagnostic criteria, targets, treatment steps, and referral thresholds.

CHRONIC DISEASE (4)

Type 2 Diabetes	Diagnosis, targets, treatment escalation	14 sections
Hypertension	Staging, when to treat, drug selection	13 sections
Dyslipidaemia	Who needs a statin and at what intensity	9 sections
Thyroid Disorders	Hypo- and hyperthyroidism	9 sections

RESPIRATORY (2)

Asthma	GINA steps, control assessment, acute attack	11 sections
COPD	Diagnosis, GOLD groups, exacerbation	8 sections

ACUTE (3)

Urinary Tract Infection	Simple, complicated, and in pregnancy	9 sections
Sore Throat	When an antibiotic is justified	7 sections
Common Cold & Sinusitis	Differentiating viral from bacterial	8 sections

SYMPTOMS (3)

Headache	SNOOP red flags and migraine management	10 sections
Low Back Pain	Triage, red flags, sciatica	6 sections
Chest Pain in Clinic	Rapid triage	8 sections

GASTROINTESTINAL (1)

IBS & GERD	Rome IV criteria and reflux management	8 sections
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HAEMATOLOGY (2)

Iron Deficiency Anaemia	Diagnosis, cause hunting, replacement	8 sections
Vitamin D Deficiency	Who to test and how to treat	6 sections

MUSCULOSKELETAL (1)

Gout	Acute attack and urate-lowering therapy	8 sections
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MENTAL HEALTH (1)

Depression & Anxiety	Screening, starting treatment, follow-up	10 sections
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PREVENTION (1)

Obesity	Assessment and stepwise management	5 sections
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WOMEN'S HEALTH (1)

Antenatal Care	Visit schedule and screening milestones	7 sections
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OLDER ADULTS (14)

Dementia	Diagnosis, subtypes, management	15 sections
Agitation in Dementia	Find the cause before reaching for a drug	10 sections
Delirium	A symptom of acute illness, not a diagnosis	13 sections
Asymptomatic Bacteriuria	The most over-treated diagnosis in older adults	12 sections
Falls	Multifactorial assessment and intervention	13 sections
Osteoporosis	Who to screen, who to treat, how	10 sections
Polypharmacy & Deprescribing	Time-to-benefit framework	11 sections
Diabetes in Older Adults	Relaxed targets, avoiding overtreatment	11 sections
Hypertension in Older Adults	Balancing benefit against falls	8 sections
Urinary Incontinence	Treatable and rarely volunteered	10 sections
Insomnia in Older Adults	Behavioural first; hypnotics are harmful	7 sections
Unexplained Weight Loss	Structured workup	9 sections
Palliative & Advance Care Planning	Questions asked early, not late	10 sections
Pressure Ulcers	Prevention, staging, treatment	10 sections

4. Reference Tools (10)

Adult Immunisation	What is given, when, and contraindications	5 sections
Preventive Screening	What to screen and at what age	7 sections
Red Flags	What must not be missed, by presentation	11 sections
Antibiotic Reference	First-line choice and duration	4 sections
Renal Dose Adjustment	Common drugs by eGFR	5 sections
Documentation Templates	SOAP, chronic review, referral, refusal	8 sections
The 10-Minute Consultation	Structuring a short visit	11 sections
Drugs to Avoid in Older Adults	Beers-based, with a safer alternative for each	4 sections
Comprehensive Geriatric Assessment	What to cover in one structured visit	8 sections

5. Prescription Templates (21)

Complete prescriptions with dose, frequency and duration, copied to the clipboard in one tap, each with its own cautions and alternatives.

RESPIRATORY (4)

Viral Upper Respiratory Infection	5 lines
Bacterial Pharyngitis	5 lines
Bacterial Sinusitis	4 lines
Mild-Moderate Asthma Exacerbation	4 lines

URINARY (2)

Uncomplicated Cystitis	4 lines
UTI in Pregnancy	4 lines

GASTROINTESTINAL (3)

Adult Gastroenteritis	5 lines
Gastro-oesophageal Reflux	3 lines
Chronic Constipation	4 lines

PAIN (3)

Mechanical Low Back Pain	6 lines
Acute Migraine	5 lines
Acute Gout	5 lines

DERMATOLOGY (2)

Atopic Dermatitis	5 lines
Superficial Fungal Infection	3 lines

CHRONIC (4)

Starting Type 2 Diabetes Treatment	6 lines
Starting Antihypertensive Treatment	5 lines
Vitamin D Replacement	5 lines
Oral Iron Replacement	5 lines

OLDER ADULTS (3)

Osteoporosis Treatment	9 lines
Chronic Pain in Older Adults	6 lines
Insomnia — Non-Drug Plan	10 lines

6. Patient Handouts (10)

Written in Arabic for the patient or family, sent by WhatsApp or printed.

How to Use Your Inhaler	~100 words
Warfarin Patient Instructions	~143 words
Sick Day Rules for Diabetes	~123 words
Managing Low Blood Sugar	~119 words
Measuring Blood Pressure at Home	~127 words
Caring for Your Lower Back	~127 words
Preventing Falls at Home	~266 words
Managing Confusion and Agitation	~312 words
My Medication Card	~129 words
Support for Caregivers	~230 words

7. Platform Capabilities

Works fully offline	A service worker caches everything after the first load; no network needed in clinic	core
Installable	Add to Home Screen gives a standalone app with its own icon	core
Unified search	One box across every section; ignores Arabic hamza and diacritics so spelling need not be exact	core
Recently used	Home screen remembers your last eight pages	optional
Dark mode	Follows the device, with a manual override that persists	optional
Copy and share	Prescriptions and templates to the clipboard; handouts to WhatsApp	core
No backend	Static files only. No server, no database, no account, no patient data transmitted	core

8. What to Consider Removing

An honest read on what earns its place, if you want to trim:

- **Strongest keep:** the Progress Note assistant, the older-adult section, red flags, prescription templates, and the drug-related tools (Beers, renal dosing, antibiotics). These are the ones that change what you do in the room.
- **Weakest keep:** Pack-Years and the HbA1c converter are single-line arithmetic you can do in your head. Anion Gap and Corrected Calcium are more inpatient than family medicine.
- **Depends on your practice:** Gestational Age and Antenatal Care matter only if you run antenatal clinics. Wells DVT is uncommon in an outpatient setting.
- **Needs local sign-off before launch:** the adult immunisation schedule and the house note format. The note skeleton reflects one department's style — anything your consultant does differently should be changed to match him.

Clinical decision support for qualified healthcare professionals. It does not replace clinical judgement. Doses and protocols are quick references that require verification against your local approved protocol and the product leaflet before use. Immunisation schedules change periodically and must be matched against the current national schedule.